Hope Hospitals	**Standard Treatment Guidelines for Neonatal Care**	
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Release Date 11/01/2017	**Created by Quality Coordinator** Signature/Name: Dr. Darpan Jakkal,	**Reviewed and Approved by Director** Signature/Name: Dr. B K Murali

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1. Objective

To ensure standardized, evidence-based, and safe care for all neonates (0–28 days) delivered or admitted at Hope Hospital.

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2. Scope

Applies to:

- **All term and preterm neonates delivered in Hope Hospital**
- **Neonates referred or admitted to nursery**
- **Paediatricians, RMOs, Nurses, Obstetricians, and Lab Services**

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3. Immediate Newborn Care (All Births)

Step	Action
Drying & Warmth	Dry with pre-warmed towel, place under radiant warmer
Airway & Breathing	Suction only if meconium-stained + non-vigorous
Cord Care	Sterile cord clamp, cut with sterile blade
APGAR Score	Record at 1 and 5 minutes
Breastfeeding	Initiate within 1 hour (ensure skin-to-skin contact)
Vitamin K	1 mg IM for >1.5 kg, 0.5 mg IM for <1.5 kg
Eye Care	Apply antibiotic eye drops (Erythromycin or Ciprofloxacin)

📋 **Document: Newborn Resuscitation Sheet, Birth Register**

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4. Neonatal Resuscitation (as per NRP 8th Edition)

Condition	Action
No Crying/Breathing	Stimulate → Clear airway → Bag & mask ventilation within 60 sec
HR < 60 bpm	Start chest compressions + ventilation 3:1
HR < 60 after 30 sec	Administer Adrenaline (0.1 ml/kg of 1:10,000 via umbilical vein)

🧪 Use neonatal-size BMV, radiant warmer, pulse oximeter, oxygen blender

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5. Neonatal Hypoglycemia

- Definition: Blood sugar <45 mg/dL
- Management:
  - Symptomatic or <25 mg/dL: 2 ml/kg 10% Dextrose IV bolus
  - Start IV glucose infusion (6–8 mg/kg/min)
  - Continue breastfeeding once stable

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6. Neonatal Jaundice

Criteria	Management
Based on AAP Nomogram	Phototherapy if bilirubin > threshold
Severe (>20 mg/dL) or Encephalopathy	Prepare for exchange transfusion

Bilirubin Monitoring	**Every 12–24 hrs in moderate to severe**

✅ **Document: Phototherapy Monitoring Chart, Bilirubin Graph**

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7. Sepsis Screening & Antibiotic Protocol

- **Signs: Poor feeding, hypothermia, lethargy, apnea, tachycardia**
- **Sepsis Screen: CBC, CRP, Blood culture**
- **Antibiotics:**
  - **First-line: Ampicillin + Gentamicin**
  - **Second-line (if no response): Cefotaxime or Meropenem**
- **Duration: 7–10 days for confirmed cases**

Document: Sepsis Score Sheet, Antibiotic Usage Log

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8. Neonatal Hypothermia

- **Axillary Temp <36.5°C**
- **Mild: Skin-to-skin contact, warm clothing**
- **Moderate/Severe: Place under radiant warmer, monitor hourly**

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9. Prematurity & LBW Management

Category	Intervention
Preterm <34 weeks	**Anticipate respiratory distress – refer if needed**
<2 kg weight	**Thermoregulation, KMC, exclusive breastfeeding**

Feeding	**NG or cup feeding; Expressed breast milk**
Supplements	**Iron, multivitamins, folic acid after 2 weeks**

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10. Immunization at Birth

Vaccine	Dose	Route	Time
BCG	0.1 ml	Intradermal	Within 24 hours
OPV-0	2 drops	Oral	Within 24 hours
Hepatitis B	0.5 ml	IM (anterolateral thigh)	Within 24 hours

Maintain: Birth Immunization Register + Mother-Child Protection Card

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11. Danger Signs in Neonates

- **Poor feeding/suck**
- **Yellow palms/soles (severe jaundice)**
- **Fast breathing (>60/min)**
- **Chest indrawing**
- **Convulsions**
- **Cold extremities or lethargy**

Immediate referral to NICU/Neonatologist if any present.

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12. Monitoring Charts & Tools

- **Neonatal Daily Chart: Temp, HR, RR, feeding, urine/stool**
- **Weight chart (daily) for LBW**
- **Fluid chart: 60–100 ml/kg/day increasing by age**
- **Drug dose chart for neonatal meds**

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13. Training Requirements

- **NRP drills for OBG & Paediatric teams – monthly**
- **Phototherapy and sepsis screening training for nursing – quarterly**
- **Immunization cold chain maintenance – reviewed weekly**

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14. Audit Indicators

Indicator	Target
Newborns receiving Vitamin K	>95%
NRP-trained deliveries	100%
Sepsis-screened symptomatic babies	100%
Appropriate phototherapy given	>90%
Hypoglycemia documentation	>90%